

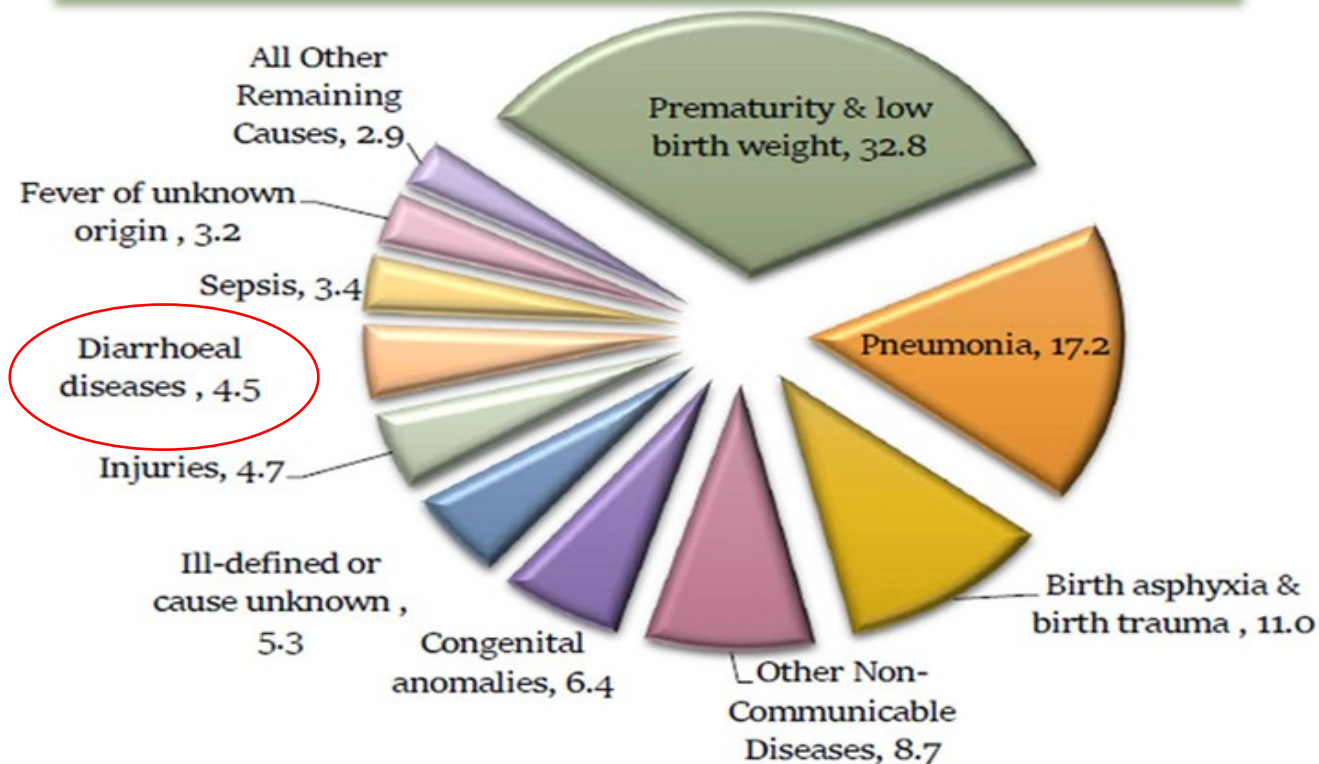


TOPIC 7 **Identification & Management** **of** **Childhood Diarrhoea**

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Chart 33 - Distribution of % proportion of deaths for age group 0-4 year in India, 2022-2024 (in %)



Childhood Diarrhoea

- Diarrhoea is a **major cause of U5 deaths** in the State
- Odisha has the 2nd highest prevalence of diarrhoea in the country

Classify Dehydration in Young Infants

For young infants, Severe and Some Dehydration share the same urgent pre-referral treatment.

SEVERE DEHYDRATION

Two or more of:

- Movement only when stimulated, or none at all
- Sunken eyes
- Skin pinch goes back very slowly

- Give first dose of IM Ampicillin AND IM gentamicin
- Refer URGENTLY to hospital; mother gives frequent ORS sips on the way

- Continue breastfeeding:

SOME DEHYDRATION

Two or more of:

- Restless or irritable
- Sunken eyes
- Skin pinch goes back slowly

- Same as Severe Dehydration: urgent referral with first antibiotic doses
- Follow Plan B
- Follow up in 2 days

NO DEHYDRATION

Not enough signs to classify as Some or Severe Dehydration.

- Give fluids and breastfeeds to treat diarrhoea at home (Plan A)
- Advise mother when to return immediately
- Follow-up after 2 days if no improvement

Classify Dehydration in Children 2 Months-5 Years

SEVERE DEHYDRATION

Two or more of:

- Lethargic or unconscious
- Sunken eyes
- Not able to drink, or drinking poorly
- Skin pinch goes back

MANAGEMENT: PLAN

Refer urgently to hospital with ORS sips on the way; continue breastfeeding

SOME DEHYDRATION

Two or more of:

- Restless or irritable
- Sunken eyes
- Drinks eagerly, thirsty
- Skin pinch goes back slowly

MANAGEMENT: PLAN

Fluid, zinc, and food for some dehydration. Follow up after 2 days if not improving.

NO DEHYDRATION

Not enough signs to classify as Some or Severe Dehydration.

MANAGEMENT: PLAN

Fluid, zinc, and food to treat diarrhoea at home. Follow up after 5 days if not improving.

Plan A: Home Management for No Dehydration

5 steps to prevent dehydration at home: 1. Extra fluids & ORS 2. Zinc for 14 days 3. Continue feeding & hygiene 4. Inform when to return 5. If SAM — refer

ORS + TYPE OF FLUID BY AGE

< 6 months	> 6 months
Breastfeed frequently (if decreased urination) + ORS + Clean water	Home available fluids + ORS

Home-available fluids: yoghurt, lemon water, rice/dal water, vegetable soup, plain water, tender coconut water.

If the child vomits, wait 10 minutes, then start again slowly — continue giving extra fluids.

AMOUNT OF ORS AFTER EACH LOOSE STOOL

Age	Amount of ORS
< 2 months	5 teaspoons full
2 months – 2 years	50–100 ml ($\frac{1}{4}$ – $\frac{1}{2}$ glass)
2 – 10 years	100–200 ml ($\frac{1}{2}$ –1 glass)
> 10 years	As much as wanted, up to 2 L/day

DIARRHOEA · PLAN A

Plan A: Zinc, Feeding & When to Return

ZINC SUPPLEMENTATION — 14 DAYS

Age	Dose
2–6 months	10 mg (½ tab)/day, dissolved with breast milk in a spoon
> 6 months	20 mg (1 tab)/day, dissolved with clean water in a spoon

Demonstrate to the mother how to prepare the zinc tablet and supervise the first dose.

CONTINUE FEEDING & HYGIENE

- Continue appropriate complementary feeding
- Promote toilet use and handwashing practice

WHEN TO RETURN IMMEDIATELY

- Child becomes sicker
- Not able to drink or breastfeed
- Develops fever
- Blood in stool
- Not able to feed

If malnutrition is present: Special care must be taken during fluid administration or refer.

DIARRHOEA • PLAN B & PLAN C

Plan B: Some Dehydration & Plan C: Severe Dehydration

PLAN B — MANAGED AT PHC/CHC BY HW,
CHO, OR MO

- Give ORS 75 ml/kg body weight within 4 hours
- If child vomits, wait 10 minutes, then give in small sips; continue breastfeeding if child wants
- Reassess after 4 hours: if no dehydration, shift to Plan A; if severe dehydration, refer for Plan C — continue ORS en route, give pre-referral antibiotics if under 2 months
- If SAM: give ORS 5 ml/kg/hr every 30 min for 2 hours, then alternate 5 ml/kg/hr ORS with starter diet hourly for next 10 hours; reassess after 4 hours

PLAN C — SEVERE DEHYDRATION

Age	First give 30 ml/kg in	Then give 70 ml/kg in
< 1 year	1 hour	5 hours
> 1 year	30 minutes	2.5 hours

Start IV immediately (Ringer Lactate; if Normal Saline unavailable). ORS can be given until IV is set up.

Monitoring: Record baseline pulse, respiration, liver span (to diagnose over-hydration). Repeat dose if radial pulse weak. Reassess every 15–20 min; give fluids more rapidly if not improving.

Dose of ORS in 4 hours

Amount of ORS as per different age to be given within 4 hours

Age	Upto 4 months	4 months -1 year	1-2 year	2-5 year	5-14 year	>15 year
Weight	<5 kg	5-8 kg	8-11 kg	11-16 kg	16-30 kg	>30 kg
Quantity of ORS	200-400 ml (1-2 glasses)	400-600 ml (2-3 glasses)	600-800 ml (3-4 glasses)	800-1200 ml (4-6 glasses)	1200-2200 ml (6-11 glasses)	2200-4000 ml (11-20 glasses)

- **Use age, if you do not know the weight**
- **If the child wants more ORS, then it can be given**
- **<6 months infants who are not breastfed, give 100ml-200ml clean water during this period along with ORS**

DIARRHOEA · PREVENTION

Prevention & Control of Under-5 Diarrhoea



PROTECT

- Exclusive breastfeeding for 6 months
- Adequate complementary feeding and supplementary nutrition



PREVENT

- Handwashing and hand hygiene
- Safe drinking water and sanitation
- Waste disposal and management
- Vaccines: Rotavirus & Measles
- Information, education & communication



TREAT

- Improved care-seeking and referral
- Case management at health facility & community level (Plan A, B, C)
- ORS and Zinc

Goal: reduce Under-5 diarrhoeal deaths through integrated protection, prevention, and prompt treatment at community level.

When to send child home & Advice



If hydration is maintained for 6 hours on ORS after rehydration.



Advice

Teach the mother how to prepare ORS solution & Zinc

Provide at least 2 packets of ORS & Zinc tablets for 14 days

Counsel on continued feeding with appropriate complementary food

Advice about handwashing, personal cleanliness and use of toilet

CLOSING SUMMARY

Key Takeaways for Field Practice



Classify before treating

Always use standard danger-sign checklists for pneumonia and dehydration before starting treatment.



Know correct dosing

Be precise with weight- or age-based dosing for Amoxicillin, Gentamicin, and ORS.



Never delay antibiotics

In severe pneumonia, antibiotics must not be delayed — even while giving bronchodilators.



Refer early

Refer severe pneumonia and severe dehydration early, with pre-referral treatment and continued ORS en route.



Educate caregivers

Counsel families on home care, danger signs, and exactly when to return immediately.